

Texas Family Caregiver Master Plan

How your cousin and her husband get paid by Texas Medicaid to care for your aunt and uncle at home, at the highest legitimate pay, with the highest level of care

Who this is for: an elderly married couple (aunt and uncle) who own their home in Texas, and their niece (the cousin) and her husband, who will move in, give up outside work, live in the home, and become the paid attendants.

What is inside: the exact Texas programs that pay relatives, 2026 dollar limits and hourly rates, an itemized step list for each of the four people, phone numbers and form numbers verified against HHSC and federal sources in September 2026, a 120-day timeline, an honest income model, the mistakes that cost families their pay, and a care-quality plan.

Read this first

The three-sentence version. Put both names on the STAR+PLUS HCBS interest list today and apply for Community Attendant Services (CAS) the same day, because CAS has no waitlist and pays for up to 50 hours a week per person. Choose the Consumer Directed Services (CDS) option so the aunt and uncle are the employers and can legally hire the niece and her husband; never make the niece the Designated Representative. When STAR+PLUS HCBS opens (typically inside a year), move to it: no 50-hour cap, 30 days of respite per person per year, home modifications, adaptive aids, and dental, all on top of attendant hours.

Prepared September 3, 2026 from Texas HHSC handbooks, the Texas Administrative Code, HHSC rate schedules, IRS, VA and SSA sources. Rules change every September 1 and every January 1; confirm any figure before you rely on it. This is a planning guide, not legal, tax or medical advice.

Contents

Contents 2

How to use this plan 4

1. The plan on one page 4

 The strategy 4

 The order of operations 5

 What is at stake (summary; full model in Section 6) 6

2. The programs, in plain English 7

 How the two doors connect 8

3. Aunt and uncle: itemized steps 9

 Phase A: This week (Days 1 to 7) 10

 Phase B: The assessment visit (Days 7 to 21) 12

 Phase C: Approval and start of pay (Days 21 to 70) 13

 Phase D: When the STAR+PLUS HCBS interest list releases (Months 3 to 12) 14

4. The cousin: itemized steps 15

 Phase A: Before you move (this week) 16

 Phase B: The assessment and hiring (Days 7 to 45) 17

 Phase C: Working (ongoing) 18

5. The husband: itemized steps 19

6. Money: getting eligible, protecting the home, and how much the caregivers earn 21

 6.1 The 2026 numbers that decide eligibility (MEPD Handbook Appendix XXXI; CMS bulletin April 27, 2026) 21

 6.2 Getting under the limits without losing anything 21

 6.3 Protecting the house from Medicaid Estate Recovery (MERP) 22

 6.4 Money the couple gets back the day Medicaid says yes 23

 6.5 What Texas pays per hour, and how much reaches the caregiver 23

 6.6 The income model: low, typical, high 24

 6.7 Taxes for the cousin and husband 26

 6.8 If the uncle (or aunt) served: the VA stack 26

7. The 120-day timeline (and what happens after) 29

8. Delivering the highest level of care 31

 8.1 Free and low-cost training for the cousin and her husband 31

8.2 What Medicaid and Medicare will buy to make care better (ask by name) 32

8.3 Respite: the relief that keeps the plan alive 32

8.4 The legal line for medical tasks (Texas Board of Nursing, 22 TAC Chapter 225) 33

8.5 The written care plan (one per spouse) 33

8.6 Oversight, complaints and protection 34

9. Pitfalls that cost families their pay (with the rule that makes each one a mistake) 35

10. Directory: who to call, what to say, where to look 37

10.1 Texas HHSC and Medicaid 38

10.2 STAR+PLUS health plans by service area (HHSC chart effective Sept. 1, 2024; unchanged as of Sept. 2026) 39

10.3 Federal, VA, legal and caregiver-support contacts 40

10.4 Forms you will hear about, by number 42

10.5 Rules to cite in a letter or appeal 43

10.6 Search terms that reliably find the official page 44

10.7 Glossary 45

How to use this plan

Each section is written for the person who has to do the work. Print the step tables for your person and check boxes as you go. Every phone number carries the words to say when someone answers. Every form has its number so you can ask for it by name.

Section	Who reads it	What you get
1. The plan on one page	Everyone	The strategy, the order of operations, and the money at stake
2. The programs, in plain English	The organizer	Which Texas programs pay relatives, and which one to use when
3. Aunt and uncle: itemized steps	Aunt, uncle, organizer	Every call, form, deadline and assessment, in order
4. The cousin: itemized steps	The cousin	Getting hired, getting the wage set high, EVV, taxes, training
5. The husband: itemized steps	The husband	Second attendant, respite provider, backup, VA role
6. Money: how much and how to get more	Everyone	Rates, hours, budget math, income scenarios, tax-free rules
7. 120-day timeline	The organizer	Week-by-week calendar with the legal deadlines that protect you
8. Highest level of care	The cousin and husband	Free training, Medicaid-paid equipment, respite, oversight
9. Pitfalls that cost families their pay	Everyone	The mistakes, with the rule that makes each one a mistake
10. Directory	Everyone	Phone numbers, websites, search terms, form numbers, rule citations

One rule for the whole plan

Pick ONE organizer (probably the cousin) who keeps a binder or shared folder for EACH spouse. Texas treats the aunt and the uncle as two separate cases with two separate assessments, two service plans, two employer accounts and two sets of hours, even though they share a home. Log every call: date, time, person's name, what they said, reference number.

1. The plan on one page

The strategy

Texas pays family caregivers through the **Consumer Directed Services (CDS)** option of its Medicaid attendant programs. The person receiving care (or their representative) becomes the legal employer, a state-contracted **Financial Management Services Agency (FMSA)** runs payroll, and the employer hires whoever they choose, including relatives, as long as the hire is not the care recipient's spouse, the employer, the Designated Representative, or those people's spouses (HHSC Form 1726, April 2026; 26 TAC Chapter 264). A niece and her husband who live in the home are eligible hires.

There are two doors into paid hours for a couple over 65 who are not yet on Medicaid. You go through both at once:

	Door 1: Community Attendant Services (CAS)	Door 2: STAR+PLUS HCBS program
What it is	State-plan attendant program run by an HHSC caseworker. Not full Medicaid (no Medicaid card), just attendant hours.	The full home-and-community waiver, run by a managed-care plan (MCO). Attendant hours plus respite, nursing, home mods, adaptive aids, meals, dental, emergency response.
Waitlist	None. Decision within 30 days of a signed application.	Interest list. As of July 31, 2026, 13,519 names, 99.9% on it under one year; HHSC released 20,595 names since Sept. 1, 2025.
Hours	Up to 50 hours/week per person (42 if "priority"). Needs a score of 24+ on Form H2060 and 6+ hours/week of need.	Not subject to the 50-hour CAS cap, and no minimum score. Hours come from the Form H2060-A task-and-minute math, bounded by a yearly cost ceiling of 202% of nursing-home cost; expect the MCO to want documentation above 50 hours.
2026 money limits	Income \$2,982/mo per person (\$5,964 couple); resources \$2,000 / \$3,000; home exempt to \$752,000 equity.	Same limits. Income over the cap is fixed with a Qualified Income Trust.
CDS (hire family)?	Yes	Yes, for attendant hours, in-home respite, nursing and therapies
Rate paid into the CDS budget (from Sept. 1, 2025)	\$16.33/hour (agency option \$17.13)	\$17.24/hour for attendant time (CDS); respite \$16.56/hour
Use it as	The bridge : paid hours within about 6 to 10 weeks	The destination : more hours, more services, respite, home modifications

The order of operations

- 1 Today:** add BOTH names to the STAR+PLUS HCBS interest list (YourTexasBenefits.com "Find Support Services", or call 844-438-5658). The request date is your place in line.
- 2 Today:** call 2-1-1 (option 2) or 877-541-7905 and open a Community Care intake for BOTH spouses, asking for Community Attendant Services with the CDS option. File Form H1200 online for each spouse. If the cousin is quitting a job to do this, the resignation must fall within 30 days before or after this application date (CCSE 2433.2), or HHSC can treat her as an available unpaid caregiver.
- 3 This week:** doctor visit for each spouse; ask the practitioner to complete **Form 3052** (Practitioner's Statement of Medical Need) and to chart every ADL limitation. Get finances under the 2026 limits. Sign powers of attorney and a HIPAA release.
- 4 Within 14 days:** caseworker home visit. Each spouse is scored on Form H2060. Describe the worst days, task by task. Say the niece will NOT keep doing tasks for free, so tasks are coded "purchased," not "caregiver."
- 5 Within 30 days:** eligibility notice (Form 2065-A) with weekly hours. Elect CDS on Form 1584, pick an FMSA, and have the aunt and uncle each be their own employer. Cousin and husband complete the FMSA hiring packet.
- 6 Weeks 6 to 10:** first paychecks. Set the wage on Form 1730 as high as the budget allows. Clock every visit in EVV.

- 7 In parallel:** if either spouse served in wartime, file for VA Pension with Aid and Attendance; if the veteran has a 70%+ service-connected rating, file for the VA caregiver stipend, which is paid on top of Medicaid.
- 8 When the interest list releases (plan on 3 to 12 months):** return Form H1200 and the plan-choice form within 30 days, get the MN/LOC and H2060 done within the MCO's 45 days, move the CDS employer accounts to the STAR+PLUS HCBS plan, add respite, home modifications and adaptive aids.

What is at stake (summary; full model in Section 6)

Scenario	Hours / week	Wage	Household caregiver income per month (gross)
Bridge: CAS, both spouses, moderate needs (35 + 35)	70	\$15.00	about \$4,550
Bridge: CAS, both spouses at the 50-hour cap, cousin and husband splitting each employer's hours	100	\$15.00	about \$6,500
Destination: STAR+PLUS HCBS, both spouses, H2060 supports 60 hours each (dementia with protective supervision)	120	\$15.50	about \$8,060
Add: VA Pension with Aid and Attendance for a married wartime veteran (paid to the veteran; must be spent on care, see 6.8)			+ up to \$2,874
Add: VA caregiver stipend if the veteran is 70%+ service-connected (paid to the cousin)			+ \$3,034 to \$3,499

Because the cousin and her husband live in the home, wages paid under a Medicaid home-and-community program are excludable from federal income tax under IRS Notice 2014-7, and Texas has no state income tax. The numbers above are therefore close to take-home apart from the 7.65% Social Security and Medicare tax. Details and the caveats (including which programs the IRS has ruled on) are in Section 6.7.

2. The programs, in plain English

Texas has several programs that will pay for an attendant. Only some of them let the family be the attendant, and they differ enormously in hours, waitlists and extras. This is the map. Rule citations are included so you can quote them to a caseworker or in an appeal.

Program	Who runs it	Who qualifies	Hours and extras	Family can be paid?	Waitlist / speed
Community Attendant Services (CAS) 26 TAC 271.81; CCSE Handbook 4600; MEPD A-4100	HHSC regional Community Care caseworker; MEPD does the money test	Income up to \$2,982/mo per person (\$5,964 couple); resources \$2,000 / \$3,000; Form 3052 medical need; H2060 score 24+; 6+ hrs/week need	Up to 50 hrs/week per person (42 if priority). Attendant tasks only. No Medicaid card.	Yes, via CDS (Form 1584). Niece and husband eligible.	No interest list. Home visit within 14 days; decision within 30 days.
STAR+PLUS HCBS (the former STAR+PLUS Waiver) 1 TAC 353.1153; STAR+PLUS Handbook	A STAR+PLUS managed-care plan (MCO) and its service coordinator; HHSC Program Support Unit	Age 21+; nursing-facility medical necessity (MN/LOC); same money limits; Qualified Income Trust if over income	Not subject to the 50-hour cap, no minimum score; bounded by the 202% cost ceiling. Plus 30 days respite/yr, nursing, therapies, \$10,000/yr adaptive aids, \$7,500 home mods, \$5,000/yr dental, meals, ERS, protective supervision hours for dementia	Yes, via CDS for attendant hours, in-home respite, nursing, therapies	Interest list; nearly everyone on it under 12 months as of July 2026. About 3 to 5 months from release to start of care.
Community First Choice (CFC) 1 TAC 354.1360-.1366	MCO (STAR+PLUS)	Only people with SSI or "SSI-related" Medicaid (income near \$994 single / \$1,491 couple, or Pickle, disabled widow(er) categories). NOT available to the 300%-income "MAO" group most retirees fall in.	No cap, no interest list; attendant hours, habilitation, ERS	Yes, via CDS	Immediate, but only if a spouse fits an SSI category. Check, then move on.
Primary Home Care (PHC) 26 TAC 271.81	MCO for STAR+PLUS members; caseworker otherwise	Full Medicaid plus same functional test as CAS	Up to 50 hrs/week. This is what CAS converts into once a person has full Medicaid.	Yes, via CDS	No interest list
Family Care (Title XX) 26 TAC 271.69; CCSE 4400	HHSC caseworker	Same income limit but resources up to \$5,000 / \$6,000; for people who fail the CAS resource test	Up to 50 hrs/week	Yes, via CDS	Regional interest list. Get on it as a backup the same day.

Program	Who runs it	Who qualifies	Hours and extras	Family can be paid?	Waitlist / speed
Day Activity and Health Services (DAHS) 26 TAC 271.77	MCO or caseworker	Chronic diagnosis; physician orders (Form 3055)	Adult day center up to 10 units/week, in addition to attendant hours	No (center staff)	Medicaid: none. Title XX: interest list
Adult Foster Care 26 TAC 271.73	Caseworker or MCO	Person cannot need "considerable" personal care	\$14.81/day (CCSE) or \$19.19 to \$67.20/day (HCBS). Cannot be combined with attendant hours.	Provider lives in; a relative is not barred	Not worth it for this family: 50 hours of CAS pays about \$117/day.
PACE (all-inclusive elder care)	PACE organization	Age 55+, nursing-home level, lives in a PACE zip code (El Paso, Amarillo, Lubbock areas)	Everything, through the PACE center	Generally no	Only if in a PACE area

The single most important eligibility fact

Most Social Security retirees have income above \$994 a month, which puts them in the "Medical Assistance Only" (MAO) 300%-of-SSI group. In that group, CFC is NOT available (STAR+PLUS Handbook 1100 and 6118), so plan on CAS now and STAR+PLUS HCBS later. If either spouse's countable income is at or under \$994 (\$1,491 combined), apply for SSI first instead: it brings full Medicaid, STAR+PLUS, and CFC immediately, and skips the interest list.

How the two doors connect

- CAS and STAR+PLUS HCBS cannot run at the same time. When HCBS starts, the caseworker ends CAS the day before MCO enrollment and the MCO re-authorizes the hours (CCSE 6430). Watch for a gap and call the Managed Care Helpline at 866-566-8989 if hours lapse.
- The same Form H2060 assessment tool is used by both, but only HCBS drops the 50-hour cap and the 24-point minimum (Form H2060 instructions; PSU Handbook 4200).
- The CDS employer accounts, the FMSA and the hired attendants can carry over; the MCO must give CDS information within 5 business days of a request (STAR+PLUS Handbook 8200).
- Both spouses can be on both programs. Each gets a separate assessment and separate hours; shared housekeeping, shopping and meal-prep minutes are scored on the "companion" range but hands-on personal care is not reduced (CCSE 2441.3).

3. Aunt and uncle: itemized steps

These steps are for the aunt and uncle, done with the organizer's help. Where a step says "each," do it twice, once per spouse, and keep two files. If either of them cannot manage paperwork, they sign HHSC **Form H1003** (Appointment of an Authorized Representative) so the organizer can speak to HHSC. That is different from a CDS Designated Representative; see the warning at Step 14.

Phase A: This week (Days 1 to 7)

#	What to do	How / where / who to call	When / how long
1	Get on the STAR+PLUS HCBS interest list, each spouse.	YourTexasBenefits.com > create account > "Find Support Services" referral; AND call HHSC at 844-438-5658 (fallback 877-438-5658). Say: "Please add [name] to the STAR+PLUS HCBS interest list and give me the request date and confirmation." Rule: 1 TAC 353.1153(b).	Day 1. Request date sets your place in line. Write it down. The record goes inactive after 120 days without a response to the annual contact.
2	Open a Community Care intake for each spouse and ask for Community Attendant Services with CDS.	Call 2-1-1, option 2 (or 877-541-7905). Say: "We want to apply for Community Attendant Services for two people in one home and be screened for every Community Care program. Please open a Form 2110 intake for each and note we want the Consumer Directed Services option." Ask for the regional CCSE office phone number.	Day 1. Home visit is due within 14 calendar days (CCSE Handbook 2300); decision within 30 days of the signed application (26 TAC 271.151).
3	File the Medicaid financial application, each spouse.	Form H1200 at YourTexasBenefits.com (or paper from the caseworker; fax 877-477-2839). On the same application, check Medicare Savings Programs (QMB/SLMB) so Medicaid pays the \$202.90 Part B premium for each spouse if eligible. Ask for a Form H1800 receipt to prove the filing date. If the couple holds well over \$3,000 in countable assets, read Section 6.2 on staging the two applications before filing the second one.	Day 1 to 3. MEPD decision clock: 45 days (MEPD B-6400).
4	Doctor visit, each spouse: get Form 3052 signed and the chart to say what they cannot do.	Ask the primary-care practitioner (MD, NP or PA) to complete Form 3052 Practitioner's Statement of Medical Need, naming the medical conditions that cause limits in bathing, dressing, toileting, transfers, walking, eating, medications, memory. Ask for a copy of the visit note. A mental-illness or dementia diagnosis alone is not enough; a physical condition causing the limitation must be documented.	Days 1 to 7. Provider must return a corrected 3052 within 5 business days when asked.
5	Get countable resources under the limit before the certification month.	Limits: \$2,000 (one applicant) or \$3,000 (both) on the first day of the month. Exempt: the home (equity to \$752,000), one vehicle, household goods, irrevocable prepaid funeral, term life. Legitimate spend-down: pay off debts, prepay funeral, home repairs, medical equipment. Do not gift money or add anyone to the deed (60-month look-back, MEPD Chapter I).	Before the month you want coverage to start.

#	What to do	How / where / who to call	When / how long
6	If income is over \$2,982 a month for either spouse, set up a Qualified Income Trust (Miller trust).	An elder-law attorney or the free Legal Hotline for Texans (800-622-2520 , age 60+) can help; HHSC has a QIT template in the MEPD Handbook (Appendix XXXVI). Keep any VA Aid and Attendance out of the trust; it is not counted as income (MEPD E-4300).	Same week; MEPD cannot certify until the trust exists.
7	Sign the legal documents while both are clearly competent.	Statutory Durable Power of Attorney, Medical Power of Attorney, HIPAA release, Directive to Physicians, and consider a Transfer on Death Deed for the home. Free Texas forms at TexasLawHelp.org. Have them notarized.	This week. Costs \$0 to \$30 for notary.
8	If either spouse is a veteran or the widow(er) of one, start the VA claim.	Texas Veterans Commission claims counselor 800-252-8387 (free), or the VA benefits line 800-827-1000 . Ask for "Pension with Aid and Attendance" (Form 21P-527EZ plus 21-2680 exam form). See Section 5 and Section 6.	This week; VA takes months, so start now.

Phase B: The assessment visit (Days 7 to 21)

#	What to do	How / where / who to call	When / how long
9	Be home for the caseworker's visit; have the cousin present as informant.	The caseworker scores each spouse on Form H2060 (0 to 3 per task) and converts minutes per task into weekly hours. Answer for the worst days. Cover every task: bathing, dressing, grooming, toileting, transfers, walking, feeding, meal prep (all meals, 7 days), medications, laundry, cleaning, shopping, escort to appointments.	Within 14 days of the intake. The visit sets the hours.
10	Make sure no needed task is coded "C" (caregiver does it free).	The service-arrangement column decides what is purchased. State clearly: "Our niece will be the paid attendant; she will not be doing these tasks unpaid." Ask that minutes be set at the top of the range the impairment score allows. Score needed: 24+ for CAS. Living with another Community Care recipient waives the 6-hour minimum (26 TAC 271.81).	At the visit.
11	Ask about "priority status" only for safety, never for hours.	Priority status (cannot transfer, eat, toilet or cook without hands-on help and no one else is present) lowers the cap from 50 to 42 hours. With live-in caregivers you almost never want it.	At the visit.
12	Ask the caseworker for the add-ons.	Home-Delivered Meals (score 20+, no means test, Title XX), Day Activity and Health Services if useful, Emergency Response Services if the caregivers will be out 8+ hours a day. Also ask to be placed on the regional Family Care interest list the same day as a backup (request date controls, 26 TAC 271.5).	At the visit.
13	Choose the Consumer Directed Services option, and get the "primary caregiver" question answered.	The caseworker must present Forms 1581, 1582, 1583, 1584 . Elect CDS on Form 1584 and pick an FMSA (both spouses can use the same FMSA; each is a separate employer). Sign Forms 1735 (employer-FMSA agreement), 1736 (orientation) and 1740 (backup plan naming the other caregiver). Form 1726 bars the "primary caregiver" in PHC/CAS/FC from being the paid attendant; Form 1583 and CCSE Handbook 2433 make that a task-by-task test, so the niece can be hired for every task coded "P" (purchased). Tell the caseworker in writing that she is not available as an unpaid caregiver, and note the 30-day job-separation rule in Section 4.	At the visit or within days after.

#	What to do	How / where / who to call	When / how long
14	Do NOT name the cousin or her husband as Designated Representative.	Form 1726: the DR and the DR's spouse cannot be paid employees of that member. If the aunt or uncle cannot handle employer duties even with FMSA support consultation (Form 1586), pick a different adult (a friend, another relative who lives elsewhere) as DR on Form 1720.	Decide before signing CDS forms.

Phase C: Approval and start of pay (Days 21 to 70)

#	What to do	How / where / who to call	When / how long
15	Watch for the eligibility notice and check the hours.	Form 2065-A arrives within 2 business days of the decision (decision due within 30 days of the signed application). It lists weekly hours and appeal rights. Ask the caseworker for the Form H2060 worksheet. Form 2101 (authorization) goes to the FMSA within 5 business days.	By about Day 30 to 45.
16	If hours look low, appeal immediately.	Request a fair hearing by the deadline printed on Form 2065-A (90 days, CCSE 2900), cite 26 TAC 271.81 and the Task/Hour Guide, and ask for a reassessment after any hospitalization, fall or decline. To keep current hours during an appeal of a reduction, the request must be made before the effective date on the notice.	Within the deadline on the notice.
17	Set the wage as high as the budget allows.	Ask the FMSA for the written budget: rate \$16.33/hour (Level 1) minus employer payroll taxes only (the FMSA's fee is billed to Medicaid separately, not taken from your budget). Set the hourly wage on Form 1730 at the maximum sustainable level; put any remainder into paid leave, holiday pay or bonuses (26 TAC 264.505). The old \$10.60 wage floor was repealed Aug. 31, 2025, so nothing but the budget limits the wage.	Before the first payroll.
18	Approve timesheets and EVV visits every pay period.	The employer (or DR) approves hours the attendant clocked through the FMSA's Electronic Visit Verification app. Unapproved or unclocked visits do not get paid.	Every pay period, forever.
19	Report changes to MEPD within 10 days and do the annual renewal.	Income or resource changes, hospital stays over 30 days, address changes. Annual reassessment of hours (CCSE 2660) and annual MEPD redetermination.	Ongoing.

Phase D: When the STAR+PLUS HCBS interest list releases (Months 3 to 12)

#	What to do	How / where / who to call	When / how long
20	Answer every HHSC call and letter.	HHSC contacts you annually to confirm interest; if they cannot reach you they mail Form H2118 . Respond within 30 days or you are removed. Between Sept. 1, 2025 and July 31, 2026, 7,466 people (44.6% of closures) were dropped for "no response."	Whenever contacted.
21	Return the release packet within 30 days.	The enrollment broker mails Form H1200 (again), Form H2053-B (health-plan choice) and the plan comparison chart within 7 days of confirming interest. Choose the MCO for your county after calling each plan's service-coordination line to ask about CDS and FMSAs. Missing 30 days means a default plan assignment.	30 days from the packet date.
22	Get the MN/LOC and the new H2060 done fast.	The MCO has 45 days to send a nurse for the Medical Necessity/Level of Care assessment (physician signature required), complete Form H2060 and write the Individual Service Plan (Form H1700-1). Ask for: attendant hours with no cap, protective supervision hours if there is memory impairment, 30 days of in-home respite (the respite worker must be someone who does not live in the home), ERS, adaptive aids, minor home modifications, dental.	Within 45 days of release (STAR+PLUS PSU Handbook 3311).
23	Move the CDS employer accounts over.	Tell the service coordinator you already use CDS and name your FMSA (it must contract with the new MCO; if not, choose another). Re-sign Forms 1584/1735 for the HCBS program. Start of care is the first day of the month after the last approval.	Realistically 3 to 5 months from release.
24	Read the approval notice and calendar the renewal.	Form H2065-D gives the start-of-care date, hours and appeal rights. The ISP and MN/LOC renew every 12 months. Request a change-of-condition reassessment after any decline.	Every 12 months.

4. The cousin: itemized steps

You are the engine of this plan: the organizer now, the paid attendant later, and the person whose documentation protects everyone's hours. Your two legal identities must stay separate: **helper with the paperwork** (Form H1003 authorized representative, medical power of attorney) is fine; **CDS Designated Representative** (Form 1720) is not, because a DR and the DR's spouse can never be paid by that employer (26 TAC 264.205; Form 1726).

The one rule that can block CAS pay, and exactly how to handle it

HHSC Form 1726 (April 2026) says a CDS service provider may not be "the primary caregiver in Primary Home Care (PHC), Community Attendant Services (CAS) or Family Care (FC)." Form 1583 (April 2026) explains what that means: "any caregiver not eligible for hire under the CCAD unmet need policy," which is CCSE Handbook 2433. That policy is **task by task**: the caseworker decides for each task whether a household member is already meeting the need (coded "C", not purchased) or whether it is unmet (coded "P", purchased). You can be hired for every purchased task; the "do not hire" list is closed (the spouse, a parent of a minor, people with APS findings, unwilling household members), and "beyond these limitations, the caseworker will not specify who cannot be hired." Two things follow. First, at the Form H2060 visit, say plainly that you are not available as an unpaid caregiver and will provide care only as the paid attendant, so tasks are coded "P". Second, **CCSE 2433.2 has a 30-day rule**: a caregiver who quits a job to provide care is treated as available for hire only if HHSC verifies she left the job within 30 calendar days before or after the application date. Time your resignation to land inside that window and keep the separation letter. STAR+PLUS HCBS and CFC have no primary-caregiver line at all.

Phase A: Before you move (this week)

#	What to do	How / where / who to call	When / how long
1	Confirm you are hireable.	18+; Social Security number and I-9 documents; a Texas DPS criminal history with no bar under Health and Safety Code 250.006; not on the Nurse Aide Registry (revoked), Employee Misconduct Registry, HHSC-OIG or federal exclusion lists. The FMSA runs these checks (Forms 1725, 1729); you do not.	Day 1
2	Get the paperwork authority, not the DR role.	Have the aunt and uncle each sign HHSC Form H1003 (Appointment of an Authorized Representative) and a HIPAA release so HHSC, the caseworker and the MCO will talk to you. Medical Power of Attorney naming you is fine too. Do not sign Form 1720 as DR.	Day 1 to 7
3	Drive Steps 1 to 8 of Section 3 for both of them.	Interest list (844-438-5658, fallback 877-438-5658, plus YourTexasBenefits referral), Community Care intake (2-1-1 option 2), Form H1200 online, doctor visits for Form 3052, financial snapshot, VA intent to file. Keep the call log.	Day 1 to 7
4	Time the job separation, and put your status in writing.	If you are leaving a job to do this, resign within 30 calendar days before or after the date the CAS application is filed (CCSE 2433.2) and keep the employer's separation letter; HHSC verifies it. Send a short letter to the regional Community Care office (via 2-1-1 or the ADRC, 855-937-2372): "I am relocating to be hired as the paid CDS attendant for my aunt and uncle. I am not available to provide unpaid care." Keep a copy for the H2060 visit.	Within 30 days of the application date
5	Make the house your legal home.	Driver's license, voter registration, mail. This is what makes the IRS Notice 2014-7 income exclusion available (you must live in the care recipient's home as your own home).	Move-in week
6	No rent. No cash. No deed changes.	Sign a written household-expense-sharing agreement (your share of utilities, groceries, repairs paid straight to the vendors). Rent to the aunt and uncle is countable income to them; gifts or paying you under the table create a Medicaid transfer penalty of \$262.37 per day.	Move-in week

Phase B: The assessment and hiring (Days 7 to 45)

#	What to do	How / where / who to call	When / how long
7	Be the informant at every assessment.	Bring a 7-day diary per spouse, the medication list, the PCP letter. For each of the 23 tasks on Form H2060 give minutes and frequency on the worst days. Ask for protective supervision hours (HCBS) if there is dementia.	Day 7 to 21
8	Complete the FMSA new-hire packet for EACH employer.	Form 1724 cover sheet, 1725 (background/registry consent), 1734 (relationship certification: every answer "No"), 1727, 1728 (workers' comp election), 1730 (wage plan), 1731 (schedule), 1732 (training log), 1733 (if delegated health tasks), 1737 (employer-employee agreement), I-9, W-4, direct deposit. Two files: one for the aunt, one for the uncle. Nobody works until the FMSA returns Form 1729 and approves the budget in writing.	Up to 3 weeks after the FMSA meeting
9	Negotiate the wage to the ceiling.	Ask the FMSA for the budget workbook. On 2026 rates the ceiling is about \$15.05/hour under CAS (\$16.33 rate) and about \$15.90/hour under STAR+PLUS HCBS (\$17.24 rate) if nothing is reserved for benefits. At least 66.36% of the budget must go to employee compensation. Put remainder into accrued paid leave, a longevity bonus, health-premium reimbursement (non-taxable) or workers' comp (26 TAC 264.505). Unused money is forfeited to the state.	Before first shift
10	Set a schedule that avoids overtime.	Overtime over 40 hours for one employer is paid at 1.5x out of the same fixed budget, which forces the hourly wage down. Best pattern: you work up to 40 hours for one spouse and the husband works up to 40 for the other; the remaining authorized hours (up to 50 each) are covered by the other person. Ask the FMSA in writing how it treats combined hours across the two employers and whether it recognizes the live-in domestic worker overtime exemption (29 CFR 552.102).	Before first shift
11	Ask the FMSA two tax questions in writing.	(a) "Do you apply IRS Notice 2014-7 to live-in caregivers and report excluded wages in W-2 Box 12 Code II?" (b) "Do you apply it to CAS/PHC pay or only to STAR+PLUS HCBS pay?" See Section 6.7.	Before first payroll

Phase C: Working (ongoing)

#	What to do	How / where / who to call	When / how long
12	Clock every shift in EVV.	Install HHAAeXchange+ (support 833-430-1307). Clock in at the start and out at the end of each authorized shift, separately for the aunt and for the uncle, never overlapping. There is no live-in exemption. Fix any missed clock within 95 days or the pay is gone. Overnight shifts split automatically at midnight.	Every shift
13	Work the authorized schedule and tasks only.	Form 1731 schedule equals the authorized units. Being present in the home is not billable; assessed tasks are. Keep a daily task log per spouse (Form 1720 service log) that matches EVV.	Every day
14	Protect and grow the hours.	Request a reassessment after any hospitalization, fall or decline. Appeal low hours within 90 days (CAS, CCSE 2900) or 60 days to the MCO (STAR+PLUS). To keep hours during the appeal: CAS, request before the effective date on Form 2065-A; STAR+PLUS, within 10 days of the MCO notice (42 CFR 438.420). Managed Care Ombudsman 866-566-8989 for MCO stalls.	As needed
15	Build the training file (Section 8).	AAA caregiver program (800-252-9240), dementia training, CPR, the CNA course if you want it. Log everything on Form 1732; it justifies the top wage and protects you in any audit.	First 90 days
16	Do the taxes right.	Your pay is subject to Social Security and Medicare tax (you earn credits) and Texas unemployment coverage (Labor Code 201.065 excludes only a spouse, a parent working for a child, or a child under 21 working for a parent). If the FMSA applies Notice 2014-7, your federal income tax on that pay is zero and you can still elect to count it as earned income for the Earned Income Tax Credit. Check your health-coverage plan: excluded pay is not in MAGI, and a household with only excluded income can fall into Texas's coverage gap (below 100% of poverty, no marketplace subsidy).	Each January
17	When the STAR+PLUS HCBS release comes, move with it.	Same FMSA if it is in the MCO's network; new Forms 1584/1735; new budget at \$17.24/hour; ask for protective supervision hours and in-home respite (delivered by an outside person, see Section 5).	Months 3 to 12

5. The husband: itemized steps

The husband is the second paid attendant, the required backup, the record-keeper, and, if either spouse is a veteran, the person who runs the VA side. Two things he cannot be: the CDS Designated Representative while his wife is paid by that employer (the DR's spouse may not be an employee), and a paid STAR+PLUS HCBS respite provider while living in the home (STAR+PLUS Handbook 7300: the in-home respite provider "must not live with the member").

#	What to do	How / where / who to call	When / how long
1	Get hired as the second CDS employee.	Same new-hire packet as the cousin (Forms 1724, 1725, 1734, 1728, 1730, 1731, 1732, 1737, I-9, W-4). Nothing in 26 TAC 264 or Form 1726 bars a niece's husband. If he will drive either person, a Texas driver's license and auto insurance card go in the file (26 TAC 264.231).	Days 21 to 45
2	Take the other spouse.	Cleanest structure: cousin is the primary attendant for one spouse, husband for the other, each up to 40 hours with that employer, and each picks up the other spouse's remaining hours (up to 50). This uses every authorized hour, avoids overtime, and keeps EVV visits from overlapping.	At scheduling
3	Be the named backup on Form 1740.	CDS requires a Service Backup Plan for critical services. He is the backup for her shifts and she for his; sick days and appointments are covered without losing hours.	At enrollment
4	If one spouse cannot be their own employer, solve the DR problem without him.	The aunt can be the uncle's DR and the uncle the aunt's (a spouse may be DR; a spouse just cannot be the paid attendant). If neither can, use a relative or friend who lives elsewhere. Whether he can be DR for the uncle while his wife is paid only by the aunt is legally arguable under 264.205 but has not been ruled on; do not rely on it.	Before CDS forms
5	Own the VA file if there is a veteran (Section 6.8).	Texas Veterans Commission 800-252-8387 for a free accredited claims advisor; VA Form 21-0966 intent to file first; physician completes 21-2680; 21P-527EZ with 21P-8416 medical expense report; enroll the veteran in VA health care (10-10EZ) and ask the VA social worker about Veteran-Directed Care, Homemaker/Home Health Aide hours and respite. If the veteran is 70%+ service-connected, file 10-10CG for the caregiver stipend with the cousin as Primary Family Caregiver and himself as Secondary.	Week 1 onward

#	What to do	How / where / who to call	When / how long
6	Keep the ledger that protects the money.	Monthly: how every VA Aid and Attendance dollar was spent (medications, supplies, equipment, private-pay care hours outside the Medicaid plan), receipts for care and home costs (future MERP deduction under 1 TAC 373.213), copies of every notice with its appeal deadline, the QIT statements if he is trustee.	Monthly
7	Run home safety and equipment.	CDC STEADI home checklist, DME orders through the PCP and MCO, minor home modification bids (\$7,500 lifetime per spouse under HCBS), ERS pendant, smoke and CO detectors, clear paths, grab bars, lighting.	First 60 days and ongoing
8	Run logistics.	Medical transportation (MCO line or 877-633-8747; 2 workdays' notice) or drive under the Individual Transportation Participant mileage program; pharmacy; DAHS or outside respite scheduling; the AAA respite voucher provider (must be someone outside the household).	Ongoing
9	Watch for burnout and schedule the breaks.	Use the STAR+PLUS HCBS respite days (delivered by an outside provider), AAA vouchers, and adult day services. Caregiver Action Network 855-227-3640; 988 any time.	Monthly
10	Keep outside income in view.	If he keeps any outside work, that taxable income keeps the household above 100% of the poverty line for marketplace health-insurance subsidies (Texas did not expand Medicaid) and may bring employer coverage. Weigh that against the paid attendant hours he can fill; the numbers are in Section 6.	Decide in month 1

Summary for the husband

The husband CAN be paid as the second attendant for CAS, PHC, CFC and STAR+PLUS HCBS attendant hours. He CANNOT be paid for STAR+PLUS HCBS respite while he lives in the home, cannot be the AAA voucher respite worker, and cannot be the DR for an employer that pays his wife. Plan around those three lines and every other hour is open to him.

6. Money: getting eligible, protecting the home, and how much the caregivers earn

6.1 The 2026 numbers that decide eligibility (MEPD Handbook Appendix XXXI; CMS bulletin April 27, 2026)

Test	One spouse applying	Both spouses applying	Notes
Monthly income cap (300% of SSI)	\$2,982	\$5,964 combined	Gross income before deductions. Over the cap? A Qualified Income Trust fixes it. VA Aid and Attendance is not counted (MEPD G-4100, E-4300).
Countable resources	\$2,000	\$3,000	Tested on the first day of each month. Both spouses' assets count even if one applies.
Home	Exempt as principal residence	Exempt	2026 equity cap \$752,000, and the cap does not apply while a spouse lives there (F-3600).
Vehicle	One, any value	One, any value	F-4221
Life insurance	Face value up to \$1,500 per person exempt	Same	Over \$1,500 face: the whole cash value counts. Term insurance is fine.
Burial	Irrevocable prepaid funeral: exempt, any value; \$1,500 designated burial fund; burial plots exempt	Same	The cleanest spend-down item.
Spousal protection when only ONE spouse is on the program	Community spouse keeps \$32,532 to \$162,660 in resources (SPRA) and an income allowance up to \$4,066.50/month	Not available: with both on the program there is no community spouse (J-1300)	This is why application timing matters; see 6.2.
Look-back	60 months	60 months	Gifts are penalized at \$262.37 per day of ineligibility (I-5100). A \$20,000 gift = 76 days.
Family Care (Title XX) backup resource limit	\$5,000	\$6,000	Same income limits; regional interest list.
SSI level (opens CFC immediately)	\$994	\$1,491	If income is this low, apply for SSI first.

6.2 Getting under the limits without losing anything

- **Take a snapshot first.** Every account, policy, title, deed, award letter, and any gift in the last 60 months. Nothing moves until you know the numbers.
- **Income over \$2,982?** A Qualified Income Trust (Miller trust) is routine in Texas: irrevocable, income only, Texas named as residuary beneficiary, deposit in the month received. HHSC publishes a model trust (MEPD Appendix XXXVI). Elder-law attorneys typically charge about \$400 to \$500. The husband can be the trustee. It can be made effective up to 3 months before the application date.

- **Resources over \$3,000?** Spend down on exempt things that help the care: irrevocable prepaid funerals for both, home repairs and accessibility work, a reliable vehicle, a hospital bed or lift. Medicare will not cover, paying off debt. **Never** gift cash or add the cousin to the deed.
- **Ask an elder-law attorney about staging.** If the couple holds more than \$3,000 but less than roughly \$165,000 in countable assets, applying for the **needier spouse first** makes the other a "community spouse" who may keep up to \$162,660 (SPRA) instead of spending down to \$3,000. Whether and when to add the second spouse is a planning decision; get it in writing. Free advice: Legal Hotline for Texans **800-622-2520, option 3** (age 60+). Referral to a paid attorney: State Bar **800-252-9690**; Texas NAELA directory at naela.org.
- **The cousin's and husband's money does not count.** Texas deems income only spouse-to-spouse and parent-to-minor (MEPD E-7100), and waiver budgets ignore in-kind support (O-1000). But **rent paid to the aunt and uncle IS countable income** to them (E-3000). So: no rent. Sign a written expense-sharing agreement and have the cousin's household pay its share of utilities, groceries and repairs **directly to the vendors**. No joint accounts with the aunt and uncle, ever.
- **Any private pay to the cousin before Medicaid starts** must be under a written, prospective Personal Care Agreement at a fair hourly wage for genuine personal care (bathing, toileting, transfers), paid by check, with daily logs. Texas treats payment for "chores a family member would normally do" (cleaning, laundry, meals, shopping, driving) as a gift even with a contract (MEPD I-4140).

6.3 Protecting the house from Medicaid Estate Recovery (MERP)

Texas recovers the cost of STAR+PLUS long-term care and CAS received at age 55+ from the **probate estate** after death (1 TAC Chapter 373; HHSC "Your Guide to MERP"). No claim is filed while a spouse survives. The cousin, as a niece, gets **no** caregiver-child exemption. These are the tools that work:

Tool	What it does	How
Transfer on Death Deed (Tex. Est. Code ch. 114) or Lady Bird deed	Passes the home outside probate. MERP claims only against the probate estate (1 TAC 373.105), so a recorded TODD keeps the home out of reach under current HHSC practice. Caveat: Est. Code 114.106 lets a personal representative reach TODD property within 2 years of death if the probate estate cannot pay claims, so keep other probate assets minimal and get attorney advice. Not a penalized transfer during life (MEPD I-3100). Revocable.	Free statutory form and instructions at TexasLawHelp.org ; both owners sign; record it with the county clerk (about \$30) before death or it is void (114.055). Do it this week while both are competent.
Care-cost deduction (1 TAC 373.213)	MERP's claim is reduced by documented costs of care that kept the person out of a nursing home, plus taxes, insurance, utilities and repairs paid by heirs. No dollar cap.	Keep a receipts binder from day one. File within 60 days of the Form 8001 notice.

Tool	What it does	How
Undue hardship waiver (1 TAC 373.209; Form 5006)	No recovery if the homestead is appraised under \$100,000 and the heirs' income is below 300% of poverty (about \$47,880 single / \$64,920 couple in 2026), but that homestead ground is only for heirs who are siblings or lineal descendants (children, grandchildren), so it does not help a niece. The other grounds (recovery would push heirs onto public assistance; other compelling reasons) are open to anyone.	Request within 60 days of Form 8001; decided in 40 days.
Automatic exemptions	No claim if the estate is \$10,000 or less, the Medicaid cost is \$3,000 or less, or there is a surviving spouse, a child under 21, a blind or permanently disabled child of any age, or an unmarried adult child who lived in the home full-time for at least a year before death.	MERP contractor 800-641-9356 ; merp@hhs.texas.gov

6.4 Money the couple gets back the day Medicaid says yes

Benefit	2026 limits	Worth
Medicare Savings Program (QMB) on the same Form H1200	Income \$1,330 single / \$1,804 couple; resources \$9,950 / \$14,910 (MEPD Q-2500, Q-1300)	Pays the \$202.90/month Part B premium for each spouse (\$4,869.60 a year for the couple) and all Medicare copays and deductibles. SLMB and QI-1 pay the premium at higher incomes (up to \$2,435 couple).
Part D Extra Help	Automatic with Medicaid or MSP; otherwise income \$23,940 / \$32,460	\$0 plan premium and deductible; copays no more than \$5.10 generic / \$12.65 brand
Full Medicaid (STAR+PLUS)	When HCBS starts	Covers what Medicare does not; STAR+PLUS HCBS adds dental, home mods, adaptive aids, respite
Over-65 homestead exemption and tax ceiling	Any income	Check the county appraisal district has the over-65 exemption and school-tax ceiling on file; Tax Code 33.06 also allows deferral of property taxes entirely.

The biggest lever

Apply for BOTH spouses. Each one who qualifies is a separate assessment and a separate set of paid hours. Under CAS that is up to 100 hours a week for the household; under STAR+PLUS HCBS the 50-hour CAS cap does not apply and hours are bounded by the 202% cost ceiling. Getting the second spouse eligible is the single biggest pay lever in this plan.

6.5 What Texas pays per hour, and how much reaches the caregiver

Since September 1, 2025 (Senate Bill 1, Rider 23, 89th Legislature) HHSC rates assume an average attendant wage of \$13.00 an hour plus 14% payroll costs. Under the agency option the agency keeps the difference. Under CDS the aunt and uncle receive the whole CDS rate into a budget the FMSA administers, the FMSA is paid its fee separately (\$120.55/month for CAS, \$210.08/month for STAR+PLUS HCBS), and the employer sets the wage. HHSC's own budget workbook requires at least 66.36% of the budget to go to employee compensation; in practice about 92% can.

Program and service	CDS rate into the budget	Agency rate (for comparison)	Wage ceiling after employer taxes*
CAS / PHC / Family Care attendant, non-priority	\$16.33/hour	\$17.13	about \$15.05
CAS / PHC / Family Care attendant, priority (42-hour cap)	\$16.51/hour	\$17.31	about \$15.20
STAR+PLUS HCBS attendant (PAS), incl. protective supervision	\$17.24/hour (\$4.31 per 15 min)	\$17.60	about \$15.90
STAR+PLUS HCBS in-home respite (outside provider only)	\$16.56/hour	\$17.56	about \$15.25
CFC attendant / CFC habilitation (only if SSI-level Medicaid)	\$17.04 / \$17.68	\$17.84	about \$15.70 / \$16.30
Support consultation (coach for the employer, from the budget)	\$26.52/hour		use briefly, if at all

*Ceiling assumes nothing reserved for benefits or bonuses; employer FICA 7.65%, FUTA 0.6% on the first \$7,000, Texas unemployment 2.7% on the first \$9,000. Source: HHSC PHC CDS budget workbook (rev. 9/1/2025) and PFD rate sheets effective 9/1/2025. The former \$10.60 base-wage rule ended August 31, 2025.

6.6 The income model: low, typical, high

Assumptions: both spouses qualify; the cousin and husband are each hired by both employers; hours are split so no one exceeds 40 hours for a single employer; wage set at \$15.00 (CAS) or \$15.50 (HCBS). Gross pay is before the 7.65% Social Security and Medicare withholding. Multiply weekly hours by 52 and divide by 12 for the monthly figure.

Scenario	Hours / week	Wage	Gross per month	Who is paid
Floor: only one spouse qualifies for CAS at 30 hours	30	\$15.00	\$1,950	Cousin alone
Bridge (CAS), both spouses, moderate needs (35 + 35)	70	\$15.00	\$4,550	Cousin 40, husband 30
Bridge (CAS), both at the 50-hour cap	100	\$15.00	\$6,500	Cousin 50, husband 50 (40 + 10 each)
Destination (STAR+PLUS HCBS), both, H2060 supports 50 each	100	\$15.50	\$6,717	Same split
Destination, high need (60 + 60 incl. protective supervision for dementia)	120	\$15.50	\$8,060	Cousin 60, husband 60 (40 + 20 each)
Destination, very high need (70 + 70; inside the 202% cost ceiling)	140	\$15.50	\$9,403	Same, or a third part-time attendant

What the Texas rules actually authorize is driven by the Form H2060 minutes, so the "typical" row is the one to plan a budget around until the assessments are done. Honest ranges for a 65+ adult needing help with 3 to 5 activities of daily living run 25 to 50 hours a week; dementia with protective supervision can push HCBS hours well above that. Every hospitalization or decline is a reassessment trigger.

Add-on	Monthly value	Notes
Federal income tax saved by IRS Notice 2014-7 (live-in) on a \$6,500/month household	about \$400 to \$500	See 6.7. Texas has no income tax. Social Security and Medicare tax still withheld.

Add-on	Monthly value	Notes
Earned Income Tax Credit if the couple elects to count excluded wages (Feigh election)	up to \$8,231/year with 3+ children; \$664 with none	Refundable; all-or-nothing election; 2026 figures (Rev. Proc. 2025-32)
STAR+PLUS HCBS extras paid to vendors, not the family, but replacing household costs	ERS \$37.61; meals; up to \$10,000/yr aids; \$7,500 home mods; \$5,000/yr dental per spouse	Section 8.2
QMB paying both Part B premiums	\$405.80	Plus deductibles and copays
VA Pension with Aid and Attendance, married wartime veteran (paid to the veteran)	up to \$2,874	Section 6.8; must be spent on care or it offsets CAS hours
VA caregiver stipend (PCAFC) if the veteran is 70%+ service-connected (paid to the cousin)	\$3,034 to \$3,499 (Level 2) or \$1,896 to \$2,187 (Level 1)	Section 6.8; overlap with Medicaid hours must be cleared with the VA
AAA Lifespan Respite voucher	about \$300/quarter guideline, often \$800 to \$1,000 per award	Paid to an outside respite provider
SNAP for the couple (elderly simplified application), CEAP utility help, Lifeline phone	SNAP up to \$546 for two; CEAP \$400 to \$1,200/yr; Lifeline \$9.25 to \$12.75/mo	Section 6.4; the cousin's wages count for the cousin's own SNAP household, not the couple's

Why this is safe money

The pay does not come from the aunt and uncle. It comes from Medicaid through the FMSA, so it never counts as a gift, never triggers a transfer penalty, and never touches their eligibility. Any money the couple pays the cousin directly must be under a written, prospective Personal Care Agreement for genuine personal care (Section 6.2).

6.7 Taxes for the cousin and husband

Question	Answer	Source
Is the pay federal-income-tax-free?	Yes, if you live in the aunt and uncle's home as your own home (no separate residence) and the pay is a "Medicaid waiver payment." STAR+PLUS HCBS pay clearly qualifies. For CAS/PHC pay the IRS says it depends on the program's nature; most FMSAs apply the exclusion to live-in CDS attendants; get the FMSA's written position and, if in doubt, a tax preparer's.	IRS Notice 2014-7; IRS Q&A (reviewed April 29, 2026), Q1 to Q4
How is it reported?	You sign the FMSA's live-in difficulty-of-care statement. W-2 Box 1 shows \$0 (or the excluded amount appears in Box 12, Code II). If any excluded pay lands in Box 1, back it out on Schedule 1, line 8s.	IRS W-2 instructions; IRS Q&A
Do I still pay Social Security and Medicare tax?	Yes, 7.65% withheld and matched by the employer budget. The family exemption covers only a spouse, parent or child. You earn Social Security credits.	26 USC 3121(b)(3); IRS Pub. 926
Can I still get the Earned Income Tax Credit?	Yes. You may elect to count all the excluded pay as earned income for the EITC and Additional Child Tax Credit. 2026 maximums: \$664 (no children), \$4,427 (1), \$7,316 (2), \$8,231 (3+).	Feigh v. Commissioner (2019); IRS AOD 2020-02; Rev. Proc. 2025-32
Texas income tax?	None. Constitutionally prohibited.	Tex. Const. art. VIII, sec. 24-a
Unemployment insurance?	Covered. Texas excludes only the employer's spouse, a parent employed by a child, and a child under 21 employed by a parent. A niece is covered. If the job ends (nursing-home placement, death), file with TWC 800-939-6631.	Tex. Labor Code 201.065
Health insurance trap	Excluded wages are not in MAGI. In Texas (no Medicaid expansion) a household whose only income is excluded pay can fall below 100% of poverty and lose marketplace subsidies entirely. Keep some taxable income in the household or plan coverage before you rely on the exclusion.	26 USC 36B(d)(2)(B); IRS premium tax credit Q&A
SNAP for the cousin's household	Counts the gross wages even though they are income-tax-free.	7 CFR 273.9(b)(1)(i)
Overtime	Over 40 hours for one employer is 1.5x and comes out of the fixed budget. Avoid it by splitting hours, or ask the FMSA whether the household live-in exemption applies (live-in 5 days/120 hours a week, written sleep and meal agreement). The 2025 federal "no tax on overtime" deduction (up to \$12,500 single / \$25,000 joint through 2028) needs the FMSA to report qualified overtime separately.	29 CFR 552.102; 29 CFR 785.22; 26 TAC 264.241
Workers' compensation	Optional in Texas; without it the employer is personally liable for injuries. Buy a policy from the CDS budget (Form 1728).	26 TAC 264.223

6.8 If the uncle (or aunt) served: the VA stack

VA money stacks on top of Medicaid, but Texas has one trap you must plan around. All figures effective December 1, 2025 through November 30, 2026.

Benefit	Who qualifies	2026 amount	How it interacts with Texas Medicaid
VA Pension with Aid and Attendance (needs-based)	65+, 90 days active duty with 1 day in a wartime period (WWII, Korea, Vietnam, Gulf War), net worth under \$163,699 (home and 2 acres excluded), non-dishonorable discharge, needs help with daily living	Veteran with spouse: \$34,488/yr = \$2,874/month ; veteran alone \$29,093; surviving spouse \$18,697 (\$1,558/mo); both spouses veterans, both A&A: \$46,143	The A&A portion is NOT counted as income by Texas Medicaid (MEPD E-4315); the base pension is counted. Trap: CCSE Handbook 2531 treats A&A as the primary fund for attendant care; unspent A&A, or A&A spent on the same tasks Medicaid covers, is converted to hours and subtracted from CAS authorizations (a full \$2,874 divided by \$16.33 = 176 hours a month). Fix: document every A&A dollar spent on medications, supplies, equipment, therapy copays and privately paid care hours OUTSIDE the Medicaid plan (nights, weekends) under a written care agreement.
Family caregiver wages as VA medical expenses	Once the veteran is A&A-rated or a physician documents custodial need; the aunt's care counts too as a household relative	Payments above 5% of the basic rate (\$1,141/yr for veteran with spouse) reduce countable income dollar for dollar	This is how a near-zero pension becomes the maximum: the couple pays the cousin under a care agreement for non-Medicaid hours and reports it on Form 21P-8416 (38 CFR 3.278). Those wages are taxable to the cousin (Notice 2014-7 does not cover VA-funded private pay).
VA Program of Comprehensive Assistance for Family Caregivers (PCAFC)	Veteran with a 70%+ service-connected rating (any era), needs in-person care 6+ months; the cousin qualifies as an extended family member and as a live-in	Monthly stipend to the cousin: Level 2 \$3,499 Houston, \$3,299 DFW, \$3,119 Austin, \$3,079 San Antonio, \$3,034 rest of Texas; Level 1 is 62.5% of that. Plus CHAMPVA health coverage, 30+ respite days, training	Paid to the caregiver, so it does not touch the couple's Medicaid budget. But 38 CFR 71.20(a)(5) bars care "simultaneously and regularly provided by or through another individual or entity," and the same person being the Medicaid-paid attendant is exactly that overlap: get the VA Caregiver Support Coordinator's written answer before relying on both. (The 70% threshold is in the 38 CFR 71.15 definition of serious injury.) Husband can be Secondary Family Caregiver.
Veteran-Directed Care	Enrolled VA patient; referral by the VA social worker; offered through Central Texas (Temple) 855-937-2372, San Antonio ACOG 888-724-8387, DFW REACH 817-870-9082, Houston Harris County AAA	A VA-funded monthly budget the veteran uses to hire and pay workers, family included	Useful for hours Medicaid will not authorize.
VA Homemaker/Home Health Aide and Respite	Enrolled VA patient with clinical need	Agency aide hours; up to 30 days/year respite	Covers the cousin's time off without unpaid gaps.

Benefit	Who qualifies	2026 amount	How it interacts with Texas Medicaid
Disability compensation (if any service-connected condition, incl. PACT Act presumptives)	Any rating	Not needs-based; A&A can be added as Special Monthly Compensation	Counted by Medicaid except its A&A portion; a 70%+ rating unlocks PCAFC and free State Veterans Home care.

- **Sequence:** Texas Veterans Commission claims advisor **800-252-8387** (free, Mon-Fri 7:30-5:30) and VA Form 21-22; file **VA Form 21-0966** Intent to File today to lock the retroactive date; physician completes **21-2680**; file **21P-527EZ** with **21P-8416** (medical expenses) and 3 months of bank statements; enroll in VA health care (10-10EZ, 877-222-8387); ask the VA social worker about Veteran-Directed Care and aide hours; if 70%+ rated, file **10-10CG**. VA status line 800-827-1000; Caregiver Support Line 855-260-3274.
- **Never** route A&A through the Qualified Income Trust (it becomes countable for the co-payment). Give HHSC the VA award letter showing the A&A breakdown; HHSC verifies quarterly with the VA (Form 1240).
- Avoid "pension poachers" who sell annuities to qualify for A&A; the 36-month VA look-back penalizes transfers at \$2,874 a month per covered dollar, and the Medicaid 60-month look-back runs alongside.

7. The 120-day timeline (and what happens after)

Dates count from the day you make the first calls. The legal clocks in the right column are the ones that protect you; write each one on a calendar the day it starts.

When	Aunt and uncle	Cousin	Husband	Legal clocks running
Day 1	Interest list, both names (844-438-5658 / YourTexasBenefits). 2-1-1 option 2: Community Care intake for both, CAS with CDS. Form H1200 online for both.	Make the calls, keep the log. Sign nothing as DR.	Locate DD-214; call TVC 800-252-8387; VA Form 21-0966 intent to file.	Interest-list request date = place in line. CCSE home visit due within 14 days. MEPD 45-day clock starts on the signed H1200.
Days 2 to 7	Doctor visits: Form 3052 signed, ADL limits charted, home-health referral if any skilled need. Financial snapshot. Sign SDPOA, MPOA, HIPAA, Directive, record the Transfer on Death Deed.	Written question to the Community Care office and two FMSAs about the Form 1726 "primary caregiver" line. Expense-sharing agreement drafted. Legal Hotline 800-622-2520.	VA net-worth inventory; physician appointment for 21-2680; 10-10EZ enrollment.	Form 3052 correction turnaround: 5 business days.
Days 7 to 21	Caseworker visit: Form H2060 for each. Worst days, every task, no "C" codes. Elect CDS (Form 1584), pick FMSA, Forms 1735/1736/1740. Ask for meals, DAHS, ERS, Family Care interest list.	Present as informant with the 7-day diaries. Start FMSA new-hire packet.	Same packet. Named backup on Form 1740.	CCSE decision due within 30 days of the signed application. QIT must exist before MEPD can certify (if income over \$2,982).
Days 21 to 45	Form 2065-A arrives with hours. Form 2101 to the FMSA within 5 business days. Set wages on Form 1730 at the ceiling. Approve the budget in writing.	FMSA orientation done; Form 1729 back; EVV training and app; first shifts.	Same. File 21P-527EZ with 21-2680 and 21P-8416.	Fair hearing on low hours: 90 days (before the effective date to keep hours). FMSA: up to 3 weeks from meeting to first workday.
Days 45 to 70	First CDS payroll. Report any change to MEPD within 10 days. MSP (QMB) approval: confirm SSA stops the \$202.90 deduction.	First paycheck (pay at least twice monthly). Register with the AAA caregiver program 800-252-9240.	Ledger started: A&A spending, receipts, notices.	EVV visit maintenance: 95 days per visit.
Days 70 to 120	Reassessment request if needs changed. Home safety fixes. Dental, DME via Medicare and MCO.	CPR class; dementia training; care plan and log in place.	Home mods bids ready for the HCBS day; NEMT set up.	MEPD redetermination annually; CCSE reassessment annually.

When	Aunt and uncle	Cousin	Husband	Legal clocks running
Months 3 to 12: interest-list release	Answer the HHSC call or Form H2118 within 30 days. Return H1200 + H2053-B within 30 days. MN/LOC + H2060 + ISP within the MCO's 45 days. Form H2065-D with start date.	Move CDS accounts to the MCO's FMSA; new budget at \$17.24. Ask for protective supervision, respite (outside provider), aids, home mods, dental.	Same hiring under the HCBS budget. PCAFC application if 70%+ rated.	Start of care = 1st of the month after the last approval. MN valid 120 days. MCO appeal 60 days, continuation within 10 days, fair hearing 120 days after MCO decision.
Every year	ISP and MN/LOC renewal (12 months); MEPD renewal; registry rechecks on employees; new Form 1730 after any rate change (next rate action expected Sept. 1, 2027).	Training log (Form 1732) and annual evaluation; tax filing with the 2014-7 exclusion.	VA Form 21P-8416 annual medical-expense report; ledger.	Interest-list annual contact if still waiting (inactive after 120 days of no response).

8. Delivering the highest level of care

Paid hours are the floor. What separates good care from great care is training, the right equipment (which Medicaid will buy if you ask by name), scheduled relief for the caregiver, and knowing the legal line for medical tasks. Everything below is free or Medicaid-paid unless a price is shown.

8.1 Free and low-cost training for the cousin and her husband

Training	Where	Cost	Why it matters
Family Caregiver Support Program (Title III-E): caregiver education, counseling, support groups, respite vouchers	Local Area Agency on Aging, 800-252-9240 . A niece caring for an aunt or uncle 60+ is an eligible caregiver (AAA Caregiver Eligibility Guide, Appendix VI).	Free	Enrolls you as the recognized family caregiver and unlocks vouchers
Dementia care: UCLA Alzheimer's and Dementia Care caregiver videos; Teepa Snow Positive Approach to Care videos; Alzheimer's Association classes and 24/7 helpline	uclahealth.org (search "caregiver training videos"); teepasnow.com; 800-272-3900	Free	Bathing, agitation, wandering, repetitive questions, safe redirection
Fall prevention: CDC STEADI caregiver brochure and "Check for Safety" home checklist	cdc.gov/steady	Free	Falls are the number one reason a home placement fails. Do the checklist before the H2060 visit and after every fall.
AARP Prepare to Care workbook	learn.aarp.org/prepare-to-care-guide	Free	Team, plan, records, self-care
CPR / AED / First Aid (American Heart Association Heartsaver or Red Cross)	Local training centers; Red Cross 800-733-2767 ; ask the AAA about free classes	About \$70 to \$120	Not required for CDS, but expected of a professional-quality caregiver
Certified Nurse Aide (Texas NATCEP): 60 classroom hours (HHSC online course is free) + 40 clinical hours + Prometric exam	hhs.texas.gov (search "NATCEP"); Prometric 800-488-5787 (new exam pricing from Sept. 1, 2026). Nursing facilities often train free in exchange for work ("train-to-hire").	Exam \$120 (\$31 written + \$89 skills) through Aug. 31, 2026; Prometric announced new pricing from Sept. 1, 2026, so confirm; tuition \$0 to \$1,500	Does not change the CDS budget, but justifies paying the top wage in it and opens agency shifts if ever needed. Lapses after 24 months without verified paid aide work.
Pressure-injury prevention basics (NPIAP); medication reconciliation checklist (AHRQ)	npiap.com; ahrq.gov	Free	Reposition every 2 hours; skin checks; one current medication list

8.2 What Medicaid and Medicare will buy to make care better (ask by name)

Item	Source	Limit	How to get it
Skilled nursing, physical/occupational/speech therapy, home health aide (while skilled care is active)	Medicare home health	\$0; must be homebound with an intermittent skilled need	Ask the PCP for a home-health referral after every hospitalization, fall or new diagnosis. Runs alongside Medicaid attendant hours.
Hospital bed, pressure mattress, wheelchair, walker, shower chair, lift, incontinence supplies	Medicare Part B and Texas Medicaid state-plan DME first; then STAR+PLUS HCBS adaptive aids	HCBS adaptive aids and medical supplies: \$10,000 per person per ISP year (MCO may exceed)	PCP writes the order with medical necessity; MCO service coordinator routes it. Never buy out of pocket first; there is no retroactive reimbursement.
Grab bars, ramps, roll-in shower, widened doors, bathroom and kitchen modifications	STAR+PLUS HCBS minor home modifications	\$7,500 lifetime per person + \$300/year repairs; done within 90 business days of the ISP	Each spouse has a separate \$7,500. Ask at the first ISP meeting.
Dentures, extractions, routine and emergency dental	STAR+PLUS HCBS dental	\$5,000 per person per ISP year ; MCO can waive for oral surgery; referral within 90 days of request	Adult Medicaid otherwise covers almost no dental. Ask.
Emergency response pendant	STAR+PLUS HCBS ERS (or CAS/CCSE ERS)	12 units per plan year	Justified for the hours the caregivers are out of the house; document that schedule.
Home-delivered meals	STAR+PLUS HCBS (no limit) or Title XX (H2060 score 20+, no means test)	Minimum 5 hot meals/week	Frees attendant minutes for hands-on care.
Nursing, therapies, cognitive rehabilitation	STAR+PLUS HCBS	No dollar cap; based on need; inside the 202% cost ceiling	Skilled nursing can also be delivered through CDS.
Rides or mileage to medical appointments	MCO transportation line; Medical Transportation Program 877-633-8747	Book 2 workdays ahead (5 days out of county)	The Individual Transportation Participant program pays a relative mileage for driving; separate from CDS wages.
Adult day center (Day Activity and Health Services)	MCO (Medicaid) or caseworker (Title XX)	Up to 10 units/week; physician order Form 3055	Structured respite; note it reduces attendant hours for those blocks.
Hospice and palliative care at home	Medicare	No STAR+PLUS disenrollment; waiver hours continue for non-duplicative needs	STAR+PLUS Handbook 3200.

8.3 Respite: the relief that keeps the plan alive

- **STAR+PLUS HCBS respite:** up to **30 days (720 hours) per person per ISP year**, in-home or out-of-home (STAR+PLUS Handbook 7300). HHSC's CDS program page lists in-home respite as a CDS-deliverable service, so the employer can hire the respite worker, but that worker **must not live with the member** (7300). Respite exists to relieve unpaid caregiving (nights, weekends), so document those unpaid hours.

- **AAA respite vouchers:** HHSC guideline \$300 per quarter; many AAAs award more. The voucher provider must be 18+ and "anyone except immediate family living in the home," so the husband cannot be the voucher worker; a friend, neighbor or agency can.
- **Texas Lifespan Respite / Take Time Texas:** HHSC's searchable respite-provider directory (respite.hhs.state.tx.us) for in-home and out-of-home providers by county.
- **Support:** AAA and Alzheimer's Association support groups (free); Caregiver Action Network help desk 855-227-3640; 988 crisis line, call or text, any hour.

8.4 The legal line for medical tasks (Texas Board of Nursing, 22 TAC Chapter 225)

An unlicensed attendant MAY do (with written RN delegation, or as a "health maintenance activity" the client can direct)	An unlicensed attendant may NEVER do
Oral, sublingual, topical, eye, ear, nose and inhaler medications; pill-organizer medications (225.11); insulin and other diabetes injections under the specific rules of 225.12; intermittent catheterization; tube irrigation; tracheostomy care and suctioning; routine oxygen; vital signs and glucose checks (225.8, 225.10). Medications through a feeding tube are prohibited except the narrow exception in 225.10(10).	Nursing assessments or care planning; dose calculations (other than measuring a prescribed liquid or splitting a pre-calculated tablet); injections other than the diabetes exceptions; taking verbal or telephone physician orders; giving the first dose of a new medication without RN documentation (225.13).

Ask the MCO nurse or the Medicare home-health RN for written delegation before doing any nursing task. It protects the cousin legally and it is what a professional agency would require.

8.5 The written care plan (one per spouse)

Keep a one-page daily care plan and a daily log for each spouse. It is good care, it is audit protection (the Form 1720 service log must match EVV), and it is the strongest evidence at the next annual reassessment.

	Checklist item
☐	Diagnoses, allergies, physicians, pharmacy, MCO service coordinator name and number
☐	Medication list with times, who sets up, who hands, who checks; RN delegation on file for anything beyond reminders
☐	Transfer and mobility method (gait belt, walker, lift), fall-risk mitigations from the STEADI checklist
☐	Bathing, skin-check and repositioning schedule; toileting and continence schedule; fluids and nutrition targets
☐	Cognition and behavior notes (what triggers agitation, what calms); wandering safeguards
☐	Who covers which hours (cousin, husband, backup on Form 1740), and the respite calendar
☐	Emergency plan: 911 criteria, hospital preference, copies of Medical POA and Directive to Physicians in the binder and on the fridge
☐	A 7-day ADL diary before every assessment, recording typical and worst days

8.6 Oversight, complaints and protection

Problem	Call	Notes
MCO slow, low hours, no assessment, service coordinator not responding	MCO member services first, then HHSC Managed Care Ombudsman 866-566-8989 (Mon-Fri 8-5)	Online form: heartbep-ext.hhs.state.tx.us . Often resolves hour disputes faster than an appeal.
HHSC eligibility (MEPD), caseworker or interest-list problems	HHS Office of the Ombudsman 877-787-8999	
Denied or reduced hours or items	STAR+PLUS: MCO appeal within 60 days , continuation of benefits if requested within 10 days , State Fair Hearing within 120 days of the MCO decision (42 CFR 438). CAS and HCBS eligibility notices (Forms 2065-A, H2065-D): fair hearing within 90 days , and before the effective date to keep services (CCSE 2900; PSU 7200).	Decision within 90 days. Appeal every low authorization.
Abuse, neglect, self-neglect or financial exploitation of an adult at home	Adult Protective Services 800-252-5400 (24/7); txabusehotline.org	Reporting is mandatory for everyone in Texas.
Complaint about a licensed home-health/attendant agency or a facility	HHSC Complaint and Incident Intake 800-458-9858	Long-Term Care Ombudsman 800-252-2412 is for facility residents only.
Medicaid fraud, or you are asked to sign for hours not worked	HHSC Office of Inspector General 800-436-6184	CDS employers and employees are audited on EVV and logs. Never bill time you are merely present.
Hospital wants to discharge before home supports are ready	Acentra Health (Texas Medicare QIO) 888-315-0636 before midnight on the discharge day	Use the "Important Message from Medicare" fast appeal; no charge while under review.

9. Pitfalls that cost families their pay (with the rule that makes each one a mistake)

Mistake	What it costs	The rule	Do this instead
Naming the cousin (or her husband) as CDS Designated Representative	Neither can be paid by that employer	Form 1726; 26 TAC 264.205	Aunt and uncle are their own employers, or each other's DR, or use an outside adult
Letting the assessor record the cousin as an available unpaid caregiver, answering "we manage fine," or quitting her job months before applying	Tasks coded "C" are not purchased; a caregiver who left work more than 30 days before or after the application date can be treated as available unpaid help	CCSE 2433, 2433.2; Forms 1583 and 1726	State that unpaid help is not available; time the resignation inside the 30-day window; the cousin is the hired attendant
Paying the cousin cash, back pay, or "under the table" before approval; adding her to the deed or accounts	Transfer penalty: \$262.37 per day of ineligibility; a \$20,000 gift = 76 days	MEPD I-1000, I-4140, I-5100	Written prospective Personal Care Agreement, checks, logs; Transfer on Death Deed instead of a deed change
Charging the cousin rent	Countable income to the couple; can push over \$2,982 and force a trust or co-payment	MEPD E-3000/E-3300	Written expense-sharing agreement, paid to vendors
Expecting Community First Choice	Not available to the 300%-income (MAO) group most retirees are in	STAR+PLUS Handbook 1100, 6118; LTSS comparison footnote 15	CAS now, STAR+PLUS HCBS later; SSI route only if income is at SSI level
Missing the HHSC interest-list contact or Form H2118	Removed from the list; start over at the bottom (44.6% of closures Sept. 2025 to July 2026 were "no response")	1 TAC 353.1153; PSU Handbook 3300	Answer within 30 days; keep address and phone current
Missing the 30-day window to return H1200 + H2053-B after release	Default plan assignment; can close the release	PSU Handbook 3311	Calendar it the day the packet arrives
Physician does not sign the MN/LOC or Form 3052, or charts only "dementia"	617 medical-necessity denials and 251 "unable to obtain MD signature" closures, Sept. 2025 to July 2026	PSU 1250; CCSE 4600	Line up a responsive PCP now; ask for physical conditions causing the limits
Accepting priority status when live-in help is present	Cap drops from 50 to 42 hours for \$0.18/hour more	26 TAC 271.81	Decline unless truly needed for safety
Overlapping or missed EVV clock-ins; billing time merely present in the home	Unpaid visits; locked after 95 days; possible fraud referral	EVV Handbook 7000, 9000, 17000	Separate clock-ins per spouse; sequential shifts; daily log
One person over 40 hours for a single employer with no overtime plan	Overtime at 1.5x eats the fixed budget and lowers the wage	26 TAC 264.241, 264.505	Split hours between the cousin and husband; ask about the live-in exemption

Mistake	What it costs	The rule	Do this instead
Counting on the husband for paid HCBS respite or the AAA voucher	Not payable to someone living in the home	STAR+PLUS Handbook 7300; HHSC respite voucher guidelines	Hire an outside respite worker; husband takes attendant hours instead
Both spouses apply at once when assets are well above \$3,000	No community-spouse protection; spend-down to \$3,000	MEPD J-1300	Ask an elder-law attorney about staging the applications (SPRA up to \$162,660)
Unspent VA Aid and Attendance	Converted to hours and subtracted from CAS authorizations	CCSE 2531	Ledger every A&A dollar to medications, supplies, equipment and non-Medicaid care hours
Depositing A&A into the Qualified Income Trust	Becomes countable for the co-payment	MEPD E-4315; Appendix XXXVI	Keep A&A out of the trust
Choosing the agency option	Attendant paid near the \$13.00 assumption; family loses control	PFD rate methodology	CDS: the family controls \$16.33 to \$17.24 per hour
Not appealing low hours, or appealing late	Hours stay low; benefits stop during appeal	CCSE 2900 (90 days, before the effective date to keep services); MCO 60 days with continuation within 10 days (42 CFR 438.420)	Appeal every low authorization; ask for continuation
Keeping a separate residence while claiming the tax exclusion	Back taxes and penalties	IRS Q&A 3 on Notice 2014-7	Move in fully; change license and voter registration
Ignoring the health-coverage cliff	No marketplace subsidy if household MAGI is below 100% of poverty in Texas	26 USC 36B	Keep some taxable income or plan coverage first
Buying equipment or home modifications out of pocket before asking	No retroactive reimbursement	STAR+PLUS Handbook 6400, 6600	PCP order, MCO service coordinator, then the waiver pays
Forgetting to record the Transfer on Death Deed, or only one spouse signing	Void; home goes through probate and MERP	Est. Code 114.055	Record with the county clerk now; both owners sign
Assuming CAS gives a Medicaid card	No drug or medical coverage; only attendant hours	MEPD A-4100	File for the Medicare Savings Program and Extra Help on the same H1200
Using stale plan lists (Aetna, BCBSTX, Amerigroup)	Wrong phone numbers; wrong plan choice	HHSC service-area chart effective Sept. 1, 2024	Use the chart in Section 10 or call 800-964-2777

10. Directory: who to call, what to say, where to look

Every number below was checked against an official page or the organization's own published materials on September 3, 2026. Numbers still change; read plan numbers off the Medicaid ID card once issued, and use 2-1-1 as the fallback for any HHSC line.

10.1 Texas HHSC and Medicaid

Who	Number / site	Hours	Say this / use it for
2-1-1 Texas (HHSC benefits line)	2-1-1, option 2; 877-541-7905 from out of state or if 2-1-1 fails	2-1-1 is 24/7	"I want to apply for Medicaid for the Elderly and People with Disabilities and Community Attendant Services for two people; please open a Community Care intake for each and note we want Consumer Directed Services." Also: application status, paper Form H1200, report changes.
YourTexasBenefits.com	yourtexasbenefits.com	24/7	File Form H1200 online, upload documents, check status, "Find Support Services" referral to the interest list. Ask for Form H1800 receipt to prove the filing date.
STAR+PLUS HCBS interest list	844-438-5658 (STAR+PLUS HCBS line, CCSE Handbook 2230, rev. June 2026); 877-438-5658 (HHSC general interest-list line) as fallback	Business hours	"Please add [name] to the STAR+PLUS HCBS interest list and confirm the request date." Update address and phone here.
Aging and Disability Resource Center (statewide)	855-937-2372 (855-YES-ADRC)	Business hours	Options counseling; help finding the regional Community Care office; Take Time Texas respite.
Area Agencies on Aging / HICAP (Texas SHIP)	800-252-9240	Business hours	Caregiver support program, respite vouchers, free Medicare/MSP/Extra Help counseling, help with Form H1200.
Texas Medicaid Enrollment Broker (plan choice)	800-964-2777; STAR+PLUS members routed to 877-782-6440	Not published	Which STAR+PLUS plans serve your county; change plans (as often as monthly).
HHS Office of the Ombudsman, Managed Care	866-566-8989; fax 888-780-8099; online heart bep-ext.hhs.state.tx.us	Mon-Fri 8-5	MCO not assessing, low hours, CDS/FMSA disputes. Call the plan first, then here.
HHS Office of the Ombudsman, general	877-787-8999	Not published	MEPD application past 45 days; caseworker or interest-list problems.
HHSC Complaint and Incident Intake	800-458-9858; tulip.hhs.texas.gov/LTCcomplaint	Mon-Fri 7-7	Complaints about licensed agencies and facilities (not MCOs).
HHSC Office of Inspector General fraud line	800-436-6184; oig.hhs.texas.gov/reportfraud	Not published	Fraud by providers; also the office that audits EVV and timesheets.
Medicaid Estate Recovery Program	800-641-9356; merp@hhs.texas.gov (contractor changes to Stellarware Sept. 1, 2026; number may change)	Not published	How CAS and STAR+PLUS HCBS costs affect the home; hardship waiver criteria; Form 5006.
TMHP (Texas Medicaid and Healthcare Partnership)	800-925-9126; LTC line 800-626-4117	Mon-Fri 7-7 CT	Medicaid ID and eligibility verification; MN/LOC form processing.
Medical Transportation Program (fee-for-service members)	877-633-8747 (877-MED-TRIP)	Mon-Fri 8-5	Rides or mileage; STAR+PLUS members call the plan's transportation line instead.

Who	Number / site	Hours	Say this / use it for
HHSC Consumer Directed Services policy	CDS@hhs.texas.gov; HHSC main 512-424-6500	Business hours	CDS rule questions; FMSA complaints that the caseworker cannot fix.
FMSA directory (CAS/PHC/FC)	resources.hhs.texas.gov/fmsa/PHC	24/7	Search FMSAs by county (26 statewide for PHC/CAS/FC); for STAR+PLUS ask the plan for its FMSA list.
HHAeXchange (EVV) Texas support	833-430-1307 ; TXSupport@hhaexchange.com	Business hours	App login, clock in/out problems, visit maintenance.
HHSC Provider Finance (rates)	737-867-7817; PFD-LTSS@hhs.texas.gov	Business hours	Current CDS and attendant rate tables.
HHSC Employability Status Check (registries)	txhhsregulatory.my.site.com/EMR	24/7	The FMSA runs Nurse Aide, Medication Aide and Employee Misconduct Registry checks here.
Adult Protective Services (Texas Abuse Hotline)	800-252-5400 ; txabusehotline.org	24/7	Abuse, neglect, self-neglect, exploitation of an adult at home. Reporting is mandatory.
Long-Term Care Ombudsman	800-252-2412	Not published	Facility residents only (rehab stays, out-of-home respite, nursing-home discharge pressure).

10.2 STAR+PLUS health plans by service area (HHSC chart effective Sept. 1, 2024; unchanged as of Sept. 2026)

Plan	Service areas	Member services
Superior HealthPlan	Dallas, Hidalgo, Lubbock, Nueces, Travis, MRSA Central, MRSA West	877-277-9772 (Mon-Fri 8-5)
Molina Healthcare of Texas	Bexar, Dallas, El Paso, Harris, Hidalgo, Jefferson, Tarrant, MRSA Northeast	866-449-6849 (Mon-Fri 8-6 CT)
UnitedHealthcare Community Plan	Bexar, Dallas, Harris, Hidalgo, Tarrant, Travis, MRSA Central, MRSA Northeast	888-887-9003; LTSS line 888-787-4107
Wellpoint (formerly Amerigroup)	Jefferson, Lubbock, Nueces, MRSA West	833-731-2160 (Mon-Fri 7-6 CT)
Community First Health Plans	Bexar (San Antonio region)	855-607-7827 / 210-358-6403
Community Health Choice	Harris (Houston region, 9 counties)	888-435-2850 / 713-295-2300 (Mon-Fri 8-5)
El Paso Health	El Paso and Hudspeth counties	833-742-3127 (Mon-Fri 8-5 MT)

Aetna Better Health, Blue Cross Blue Shield of Texas and FirstCare do not hold STAR+PLUS contracts even though an older HHSC logo page still shows them. Choose the plan by calling each service-coordination line and asking: "Which FMSAs are in your network? How fast do you complete the H2060 for new members? Do you authorize protective supervision?" You can switch plans monthly.

10.3 Federal, VA, legal and caregiver-support contacts

Who	Number / site	Hours	Use it for
Social Security Administration	800-772-1213 (TTY 800-325-0778)	Mon-Fri 8-7	Benefit verification letters for the H1200; Extra Help; the cousin's wage record.
Medicare	800-633-4227 (TTY 877-486-2048)	24/7	Home health, DME, hospice; confirm Part B premium stops after QMB.
IRS	800-829-1040; irs.gov Notice 2014-7 Q&A	Mon-Fri 7-7 local	Difficulty-of-care exclusion; W-2 Box 12 Code II; EITC election.
U.S. Department of Labor, Wage and Hour	866-487-9243	Business hours	Live-in domestic worker overtime rules.
Texas Workforce Commission (unemployment)	800-939-6631; twc.texas.gov	Mon-Fri 8-6 CT	Claims if CDS employment ends.
Texas Veterans Commission / Texas Veterans Call Center	800-252-8387 (800-252-VETS)	Mon-Fri 7:30-5:30 CT	Free accredited claims advisor for pension, A&A, PCAFC.
VA Benefits (pension status)	800-827-1000 (the older 877-294-6380 no longer appears on va.gov)	Mon-Fri 8am-9pm ET	Pension and A&A claim status; Pension Intake Center, PO Box 5365, Janesville WI 53547-5365.
VA Caregiver Support Line	855-260-3274	Mon-Fri 8-8 ET	PCAFC (Form 10-10CG), Caregiver Support Coordinator, respite.
VA Health Benefits (enrollment)	877-222-8387	Mon-Fri 8-8 ET	Form 10-10EZ; find the VA medical center; Veteran-Directed Care referral via the social worker.
Legal Hotline for Texans (Texas Legal Services Center)	800-622-2520, option 3; texaslawhelp.org	Mon-Fri 8:30-5	Free advice for 60+: POA, TODD, Medicaid, MERP. Free forms at TexasLawHelp.org.
Texas RioGrande Legal Aid / Lone Star Legal Aid / Legal Aid of NorthWest Texas	833-329-8752 / 800-733-8394 / 888-529-5277	Business hours	Free civil legal aid by region.
State Bar of Texas Lawyer Referral	800-252-9690; texasbar.com/lris	Mon-Fri 8:30-4:30	Elder-law attorney for the QIT, care agreement, application staging. Also the Texas NAELA directory at naela.org.
Alzheimer's Association	800-272-3900	24/7	Dementia care coaching, classes, support groups.
Caregiver Action Network help desk	855-227-3640	Mon-Fri 7-6 CT	Caregiver coaching.
988 Suicide and Crisis Lifeline	Call or text 988	24/7	Caregiver crisis or burnout.
Eldercare Locator	800-677-1116	Business hours	National referral to the local AAA/ADRC and Veteran-Directed Care sites.
Acentra Health (Texas Medicare QIO)	888-315-0636	Business hours	Fast appeal of a hospital discharge before home supports are ready.
Prometric (Texas nurse aide exam)	800-488-5787	Business hours	CNA written and skills exam scheduling.

Who	Number / site	Hours	Use it for
Texas Lifeline (phone discount)	866-454-8387	Business hours	Lifeline application once Medicaid or SNAP is approved.
Texas Comptroller property tax help	800-252-9121; comptroller.texas.gov	Business hours	Over-65 homestead exemption (Form 50-114), tax ceiling, deferral (Form 50-126).

10.4 Forms you will hear about, by number

Form	What it is	Who handles it
H1200 / H1200-EZ	Medicaid (MEPD) application; EZ version for Medicare Savings Programs only	Aunt and uncle (organizer); YourTexasBenefits.com
H1003	Appointment of an Authorized Representative for HHSC dealings	Aunt and uncle name the cousin
H1020	HHSC request for missing information; final due date is day 39	Answer immediately
H1746-A	MEPD referral cover sheet routing the H1200 and waiver status	HHSC staff
3052	Practitioner's Statement of Medical Need (CAS/PHC)	The doctor
2110 / 2060 (H2060) / H2060-A / H2060-B	Community Care intake; the needs assessment and task/hour guide that sets weekly hours; STAR+PLUS addenda	Caseworker or MCO nurse, with the family as informant
2101 / 2065-A	Authorization of Community Care services; notice of decision and hours (appeal rights)	Caseworker; watch the mail
1581 / 1582 / 1583 / 1584	CDS overview; CDS responsibilities and self-assessment; employee qualification requirements; choice of CDS, SRO or agency and the FMSA	Aunt and uncle sign
1720 / 1721	Appointment / revocation of a Designated Representative (never the cousin or husband)	Aunt and uncle
1722	EVV responsibility option (1, 2 or 3)	Employer
1724 / 1725 / 1729	New-hire packet cover; background and registry check consent; FMSA applicant verification (no work before it is signed)	Cousin, husband, FMSA
1726 / 1734	Relationship definitions and certification of relationship status (every answer "No")	Employer and each employee
1728	Liability acknowledgment / workers' compensation election	Employer
1730 / 1731 / 1732 / 1733 / 1737	Wage and benefits plan; work schedule; training log; nursing-licensure exemption for delegated tasks; employer-employee agreement	Employer and employee
1735 / 1736 / 1740	Employer-FMSA service agreement; orientation record; service backup plan	Employer and FMSA
H2053-B / H3676 / H1700-1 / H2065-D / H2118	STAR+PLUS plan choice; pre-enrollment assessment authorization (45-day clock); Individual Service Plan; notice of managed-care services (start date, appeal rights); interest-list continued-interest confirmation (30 days)	Enrollment broker, MCO, HHSC Program Support Unit
MN/LOC	Medical Necessity and Level of Care assessment (TMHP portal), physician-signed; sets the 202% cost ceiling	MCO nurse and the doctor
H1272 / H1275	Spousal resource assessment; expanded SPRA request	MEPD specialist, if staging applications
8001 / 5006	MERP notice of intent to file a claim; hardship waiver application (60 days)	Estate representative, later

Form	What it is	Who handles it
1240	HHSC request to the VA for the benefit breakdown (A&A vs base pension)	MEPD specialist
H1010 / H0011	SNAP application; Texas Simplified Application Project for all-elderly households	Aunt and uncle
50-114 / 50-126	Homestead exemption application; over-65 tax deferral affidavit	County appraisal district
VA 21-0966 / 21-22 / 21P-527EZ / 21-2680 / 21P-8416 / 21P-0969 / 10-10EZ / 10-10CG / 10-10EC	Intent to file; VSO representation; pension application; A&A physician exam; medical expense report; income/asset statement; VA health enrollment; PCAFC; extended-care copay	Veteran, with TVC advisor
IRS W-2 Box 12 Code II; Schedule 1 line 8s	Where excluded Medicaid waiver pay is reported	FMSA and the cousin's tax preparer

10.5 Rules to cite in a letter or appeal

Citation	What it governs
1 TAC 353.1153	STAR+PLUS HCBS eligibility, interest list, 202% cost limit
1 TAC 354.1360-354.1366	Community First Choice
1 TAC 355.7052 (and repealed 355.7051)	Attendant rate methodology; former \$10.60 base wage
1 TAC Chapter 357	Fair hearings
1 TAC Chapter 358 (358.433, 358.437)	MEPD financial eligibility; special income limit; couple budgets
1 TAC Chapter 373 (373.105, 373.209, 373.213)	Estate recovery: scope, hardship waivers, deductions
26 TAC Chapter 264 (264.205, 264.223, 264.225-.233, 264.241, 264.403, 264.505-.511)	Consumer Directed Services: DR, liability, hiring checks, overtime, FMSA transfer, budgets
26 TAC Chapter 271 (271.5, 271.53, 271.69, 271.71, 271.77, 271.81, 271.151)	Community Care eligibility: interest lists, income, Family Care, meals, DAHS, PHC/CAS, case timelines
22 TAC Chapter 225	What an unlicensed attendant may do under RN delegation
Tex. Est. Code ch. 114; 752.051; 1104.202; 1357	Transfer on Death Deed; durable POA; declaration of guardian; supported decision-making
Tex. Health & Safety Code 166.154, 181.154, 250.006	Medical POA; HIPAA authorization; criminal-history bars
Tex. Labor Code 201.065; Tex. Tax Code 11.13, 11.26, 33.06	Unemployment family exclusion; homestead exemptions, ceiling, deferral
IRS Notice 2014-7; IRC 131; 26 USC 3121(b)(3); Feigh v. Commissioner, 152 T.C. No. 15	Difficulty-of-care exclusion; FICA family exemptions; EITC election
29 CFR 552.102; 29 CFR 785.22	Live-in domestic worker overtime exemption; sleep time
38 CFR 3.274-3.278; 38 CFR 71.15-71.40; 38 U.S.C. 1720G	VA pension net worth, transfers, medical expenses; PCAFC
Handbooks: MEPD (A-4100, B-6400, E-3000, E-4300, E-7100, F-3600, F-4200, G-4100, G-6200, I-3100, I-4140, I-5100, J-1300, O-1000, Q-2500, App. XXXI, XXXVI); CCSE (2300, 2400, 2531, 2900, 3200, 4400, 4600, 6300); STAR+PLUS (1100, 3200, 3400, 6100-6600, 7300, 8200); PSU (1200, 3311, 4200, 7200); EVV (7000, 9000, 17000)	All at fhhb.hhs.texas.gov/handbooks

10.6 Search terms that reliably find the official page

- "STAR+PLUS HCBS interest list" hhs.texas.gov; "interest list reduction" hhs.texas.gov; "1 TAC 353.1153"
- "Community Attendant Services" CCSE Handbook 4600; "26 TAC 271.81"; "Form 3052 practitioner's statement of medical need"
- "Form H2060 needs assessment questionnaire task hour guide"; "CCSE Handbook 2441.3 companion case"
- "Consumer Directed Services" "How CDS Works" hhs.texas.gov; "CDS FAQ" hhs.texas.gov pdf; "Form 1726 relationship definitions CDS"; "26 TAC 264.205 designated representative"; "Form 1730 wage and benefits plan"; "PHC CDS budget workbook"
- "FMSA agencies" resources.hhs.texas.gov/fmsa; "CDS support advisors" resources.hhs.texas.gov
- "9-1-2025 PHC rates" pfd.hhs.texas.gov; "9-1-2025 STAR+PLUS rates" pfd.hhs.texas.gov; "Rider 23 attendant wage \$13.00 Information Letter 2025-11"
- "MEPD Handbook Appendix XXXI budget reference chart"; "MEPD Appendix XXXVI qualified income trust"; "MEPD I-5100 transfer of assets divisor"; "MEPD I-4140 compensation"; "MEPD J-1300 spousal definitions"
- "Your Guide to the Medicaid Estate Recovery Program" hhs.texas.gov; "1 TAC 373.213"; "Transfer on Death Deed kit" texaslawhelp.org
- "STAR+PLUS Handbook 7300 respite"; "STAR+PLUS Handbook 6400 adaptive aids"; "STAR+PLUS Handbook 6600 minor home modifications"; "STAR+PLUS Handbook 8200 consumer directed services"
- "EVV policy handbook 9000 visit maintenance 95 days"; "HHAeXchange Texas CDS employer"
- "STAR+PLUS health plans by service area" pdf hhs.texas.gov; "Ombudsman managed care help" hhs.texas.gov
- "certain Medicaid waiver payments may be excludable from income" irs.gov; "Feigh v. Commissioner AOD 2020-02"; "Fact Sheet 79B live-in domestic service workers" dol.gov
- "veterans pension rates" va.gov; "38 CFR 3.278"; "CCSE Handbook 2531 aid and attendance"; "MEPD E-4315 VA benefits"; "PCAFC eligibility" caregiver.va.gov; "Texas Veterans Commission claims"
- "Area Agencies on Aging" hhs.texas.gov; "respite voucher program guidelines" hhs.texas.gov; "Take Time Texas respite"; "NATCEP" hhs.texas.gov; "22 TAC 225 delegation"

10.7 Glossary

Term	Meaning
ADL / IADL	Activities of daily living (bathing, dressing, toileting, transfers, eating, walking) / instrumental activities (meals, cleaning, shopping, medications, escort)
A&A	VA Aid and Attendance, the highest tier of VA pension or a compensation add-on for people who need help with daily living
CAS / PHC / FC	Community Attendant Services (Medicaid-funded attendant hours without full Medicaid) / Primary Home Care (same, for people with full Medicaid) / Family Care (state-funded backup)
CCSE	Community Care Services Eligibility, the HHSC caseworker unit that runs CAS, PHC, FC, meals, DAHS
CDS / FMSA / DR	Consumer Directed Services (the person is the employer) / Financial Management Services Agency (payroll agent) / Designated Representative (runs employer duties; cannot be paid or married to an employee)
CFC	Community First Choice, an attendant benefit with no cap and no waitlist, available only to SSI-level Medicaid members
EVV	Electronic Visit Verification: the required clock-in/clock-out for every attendant shift
H2060 / MN/LOC / ISP	The needs assessment that sets hours / the medical necessity and level-of-care assessment for STAR+PLUS HCBS / the Individual Service Plan (Form H1700-1)
MAO	Medical Assistance Only: the Medicaid group for people with income between the SSI level and 300% of SSI; most retirees
MCO / service coordinator	The STAR+PLUS managed-care plan and its nurse or case manager who assesses and authorizes services
MEPD	Medicaid for the Elderly and People with Disabilities, the HHSC financial eligibility unit
MERP	Medicaid Estate Recovery Program
MSP / QMB / SLMB / QI	Medicare Savings Programs that pay Medicare premiums and, for QMB, all cost-sharing
PAS	Personal assistance services: attendant hours
PCAFC	VA Program of Comprehensive Assistance for Family Caregivers (monthly stipend to the caregiver)
PSU	HHSC Program Support Unit that processes STAR+PLUS HCBS eligibility
QIT	Qualified Income Trust (Miller trust) used when income is over the \$2,982 cap
SPRA / MMMNA	Spousal Protected Resource Amount / Monthly Maintenance Needs Allowance for a community spouse
STAR+PLUS HCBS	The Texas home-and-community-based waiver program for adults who meet nursing-facility level of care (formerly the STAR+PLUS Waiver)
TODD	Transfer on Death Deed

Sources: Texas HHSC handbooks and forms at fhhb.hhs.texas.gov (MEPD, CCSE, STAR+PLUS, PSU, CDS, EVV), HHSC Provider Finance rate schedules effective September 1, 2025, HHSC interest-list data through July 31, 2026, Texas Administrative Code, CMS Informational Bulletin of April 27, 2026, IRS Notice 2014-7 and Q&A, VA pension rate tables effective December 1, 2025, OPM 2026 salary tables, Texas statutes. Research and cross-checking completed September 3, 2026. Rules change on September 1 and January 1 each year.